

Chemotherapy for bladder cancer

Chemotherapy uses anti-cancer (cytotoxic) drugs to destroy cancer cells. Chemotherapy into a vein can be used to treat muscle-invasive, locally advanced and advanced bladder cancer.

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About chemotherapy for bladder cancer

Chemotherapy uses anti-cancer (cytotoxic) drugs to destroy cancer cells. Cytotoxic means toxic to cells. How this treatment is used to treat [bladder cancer](#) depends on the stage of the cancer.

Chemotherapy for non-muscle-invasive bladder cancer can be given directly into the bladder. This is called intravesical chemotherapy. We have separate information about [intravesical chemotherapy](#).

The information on this page is about chemotherapy given into a vein (intravenous).

Chemotherapy into a vein can be used for:

- muscle-invasive bladder cancer – this means the cancer has spread into or through the muscle layer of the bladder wall
- locally advanced bladder cancer – this means the cancer has spread outside the bladder into nearby areas of the body.

You may have chemotherapy into a vein:

- before [surgery](#) or [radiotherapy](#), to shrink the cancer and reduce the risk of it coming back – this is called neo-adjuvant chemotherapy
- with radiotherapy, to make treatment work better – this is called [chemoradiation](#)
- after surgery, if there is a high risk of the cancer coming back – this is called adjuvant chemotherapy.

Chemotherapy into a vein is also used to treat advanced bladder cancer. Advanced means the cancer has spread to other parts of the body. You may have chemotherapy on its own as your main treatment to help to shrink and control the cancer and improve your symptoms. It may help you live for longer.

We have more information about [coping with advanced cancer](#).

Chemotherapy drugs for bladder cancer

When chemotherapy is given into a vein to treat bladder cancer, most people have a combination of drugs. Some common combinations are:

- [gemcitabine and cisplatin \(GemCis\)](#).
- [methotrexate, vinblastine, doxorubicin and cisplatin \(MVAC\)](#).
- [mitomycin and fluorouracil \(5-FU\)](#) – usually given at the same time as radiotherapy (chemoradiation)
- [gemcitabine](#) – usually given at the same time as radiotherapy (called chemoradiation)
- [paclitaxel and carboplatin](#)
- [gemcitabine and carboplatin \(GemCarbo\)](#).

Other chemotherapy drugs may also be used, or you may have one of these drugs on its own.

We have more information on side effects of other drugs in our [A-Z of treatments](#).

How chemotherapy is given

Your cancer doctor or nurse will explain your treatment plan and what to expect. You usually have treatment as an outpatient in the chemotherapy day unit. You can usually go home afterwards. Sometimes, you may need to stay in hospital for 1 to 2 days.

We have more information about [chemotherapy](#) and [chemotherapy side effects](#). Or you can find more about specific chemotherapy drugs in our [A-Z of treatments](#).

About our information

References



Below is a sample of the sources used in our bladder cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Mottet N, Bellmunt J, Briers E, et al. Non-muscle-invasive bladder cancer (TaT1 and CIS). European Association of Urology (Internet), 2021. Available from uroweb.org/guideline/non-muscle-invasive-bladder-cancer (accessed September 2021).

Witjes JA, Bruins HM, Cathomas R, et al. Muscle-invasive and metastatic bladder cancer. European Association of Urology (Internet), 2021, Available from uroweb.org/guideline/bladder-cancer-muscle-invasive-and-metastatic (accessed September 2021).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

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⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help